NCCP 00892 · VERSION 2

Adjuvant ribociclib + endocrine therapy

The usual adjuvant starting dose is ribociclib 400 mg once daily on Days 1–21, followed by Days 22–28 off, for up to three years. The aromatase inhibitor continues every day on its own plan; ovarian suppression is added when indicated.

> Discussion aid: review with the patient and complete the approved local consent process.

Purpose and plan

☐ Exact indication, treatment aim and uncertainty explained

☐ Expected duration and review / switch points discussed

☐ Alternatives and the option not to proceed discussed

☐ Patient priorities, capacity and support needs explored

Practical safety

☐ Medicine, route, schedule and missed-dose instructions

☐ Menopause and ovarian-suppression requirements if relevant

☐ Pregnancy, contraception / fertility and breastfeeding

☐ Monitoring, symptom reporting and contact plan

Selected material risks

☐ Low white cells and infection

☐ QT prolongation / heart-rhythm problems

☐ Rare serious lung or skin reactions

☐ Joint and muscle symptoms

☐ Bone loss and fractures

☐ Liver injury / abnormal blood tests

☐ Nausea, fatigue and hair thinning

☐ Hot flushes and menopausal symptoms

☐ Vaginal dryness and sexual effects

☐ Injection-site symptoms if ovarian suppression is prescribed

Discuss the treatment aim, alternatives, benefit and uncertainty, important medicine-specific risks, reproductive precautions, symptom support and planned duration. Confirm the endocrine partner and any ovarian-suppression plan. The timeline does not authorise a restart or certify clinical review.

Individual benefit, alternatives and concerns

Agreed checks and next review

FBC, renal and liver profile; potassium, calcium, magnesium and phosphorus. ECG before initiation; QTcF must be below 450 ms. Correct electrolytes and review QT/CYP3A interactions. Confirm stage, recurrence-risk criteria, prior treatment, AI and ovarian-suppression plan; reconcile the NCCP node-negative eligibility/exclusion conflict before selection. FBC, renal and liver profile every two weeks for cycles 1–2, then before cycles 3–6, then as clinically indicated. Electrolytes before each of the first six cycles, then as clinically indicated. Repeat ECG around cycle 1 Day 14, then as clinically indicated. Increase liver monitoring for grade 2 or worse abnormalities; review fever, jaundice, palpitations/fainting, breathlessness and severe skin symptoms. Keep early-breast surveillance and bone/endocrine follow-up distinct from treatment-toxicity checks and from metastatic response imaging.

Clinician / date: _____ Patient discussion documented: _____

Consent is an ongoing conversation and may be reconsidered. This aid is not treatment authorisation.

SACTCheck · Developed by Dr Paul O'Brien · v0.84.21 · Sources checked 2026-10-25
Source reconciled; independent clinical, pharmacy and patient/PPI review pending